Machine Translated by Google

Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
PRESIDENTIAL EBOLA TASK FORCE 17



MVE17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°158 /MVE-BDBV/19/10/2026

Report date: October 19, 2026 Publication date: October 20, 2026

HIGHLIGHTS (last 24 hours)

- Eighty-five (85) new confirmed cases of Ebola virus disease (EVD), including 29 community deaths, have been reported: Ituri (43), North Kivu (33), Haut-Uélé (6), Tshopo (2), and North Ubangi (1). In addition, 15 more deaths of confirmed cases were recorded in Ebola Treatment Centers (ETCs). No new health zones have been affected in the last 24 hours.
- A total of 514 new samples were analyzed; 85 of these samples tested positive for virusBundibugyo, representing a 16.5% positivity rate

ACCUMULATIVE CAS	ACCUMULATIVE DEATH CONFIRMED	LETHALITY	PATIENTS IN ISOLATION/CTE	ACCUMULATIVE HEALED	TRACKING RATES CONTACTS (Of the Day)
9637	4 646	48,2%	1029	2 574	77,8%

Affected provinces and affected health zones

8 Provinces affected

Uele, Ituri, North Kivu, South Kivu, Tshopo, Lower Uele and Sud Ubangi

66 Health Zones affected out of 178(37,1%)

Summary of key points from the last 24 hours

o As of October 19, 2026, 85 new confirmed cases of the disease Ebola virus (EV) cases have been reported in five provinces: Ituri (43 cases), the North Kivu (33 cases), Haut-Uélé (6 cases), Tshopo (2 cases) and the North Ubangi (1 case). Among these cases, 29 community deaths were reported. In addition, 15 confirmed deaths were recorded in the CTEs (10 in Ituri, 4 in North Kivu and 1 in Haut-Uélé) bringing to 44 the total number of confirmed deaths notified during the period.

o In addition, 24 patients (5 in Ituri, 10 in North Kivu, 7 in Haut-Uélé and 2 in (Tshopo) were declared cured after obtaining two negative results in the follow-up test for Ebola Virus Disease.

o Since the start of the epidemic, the DRC has accumulated 9,637 confirmed cases and 4 646 deaths, representing a case fatality rate of 48.2%. Ituri remains the epicenter of the epidemic, with 72.7% of cumulative confirmed cases and **50.6%** of new confirmed cases reported in the last 24 hours

o 66 out of 178 health zones (37.1%), spread across eight provinces, were affected since the beginning of the epidemic.

Machine Translated by Google

Table 1. Distribution of confirmed cases and deaths by affected province, as of October 19, 2026.

Province	New cases confirmed (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	43	7003	3 242	46,3%	28/36 (77,8 %)
North Kivu	33	2117	1 191	56,3%	17/34 (50,0 %)
Haut-Uélé	6	421	180	42,8%	7/13 (53,8 %)
Boss	2	75	26	34,7%	7/23 (30,4 %)
South Kivu	0	3	1	33,3%	1/34 (2,9 %)
Bas-Uélé	0	13	4	30,8%	4/11 (36,4%)
Sud Ubangi	0	2	1	50,0%	1/16 (6,3%)
Nord Ubangi	1	3	1	33,3%	1/11 (9,1%)
Total	**85**	**9 637**	**4 646**	**48,2%**	**66/178 (37,1%)**

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 19, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	**7003**	**3242**	**46,3%**	**43**	**15**	**10**	**25**
Give	11	1	9,1%				0
Understand	13	6	46,2%	1	0	0	0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	216	48	22,2%				0
have	2	2	100,0%				0
Bunia	1905	638	33,5%	10	3	2	5
Ladies	41	10	24,4%				0
Drodro	15	7	46,7%				0
Fireworks	84	31	36,9%				0
Gethy	11	7	63,6%				0
Flounder	2	0	0,0%				0
Kilo	38	12	31,6%				0
Team	191	115	60,2%	3	2	0	2
Liter	313	158	50,5%	4	2	0	2
Logo	14	9	64,3%				0
Fight	36	9	25,0%				0
distributed	4	1	25,0%				0
Washing	74	32	43,2%				0
Darkness	347	176	50,7%	10	3	3	6
Mangala	382	164	42,9%				0
Mongbwalu	716	345	48,2%	1	1	1	2

Machine Translated by Google

Nia -Nia	333	193	58,0%	2	2	0	2
Nizi	832	301	36,2%	5	0	3	3
Legumes	134	41	30,6%				0
Jungle	19	5	26,3%				0
Rwampara	1171	406	34,7%	4	2	1	3
Tchomia	79	25	31,6%	3	0	0	0
A valves	THAT	493	THAT				0
North Kivu	**2117**	**1191**	**56,3%**	**33**	**11**	**4**	**15**
Me	537	359	66,9%	13	4	1	5
Farm	24	8	33,3%				0
Butembo	315	188	59,7%	3	0	0	0
Ten	1	0	0,0%				0
Kalunguta	79	31	39,2%	0	0	1	1
Cut	647	395	61,1%	3	1	1	2
Source	3	2	66,7%				0
Kyondo	44	21	47,7%				0
Lubero	36	14	38,9%	1			0
Be careful.	79	31	39,2%	4	0	1	1
Alimbong	5	3	60,0%				0
Manguredjipa	16	5	31,3%				0
Masereka	39	13	33,3%				0
The museums	154	72	46,8%	2	1	0	1
Mutwanga	29	9	31,0%	5	4		4
Oicha	54	24	44,4%	2	1	0	1
Vuh	55	16	29,1%				0
Upper Uele	**421**	**180**	**42,8%**	**6**	**2**	**1**	**3**
Government Mangbetu	45	19	42,2%				0
Dumb	1	1	100,0%				0
Gombari	3	1	33,3%				0
Mathematics	108	46	42,6%				0
Lining	106	59	55,7%	2	2	1	3
Beans	6	3	50,0%	1	0	0	0
Ordinary	152	51	33,6%	3			0
Boss	**75**	**26**	**34,7%**	2	1	0	1
Bafwasende	17	2	11,8%				0
Kabondo	10	2	20,0%	1	1	0	1
Lubunga	1	0	0,0%				0
Maquis - Kisangani	33	14	42,4%				0
Mangobo	10	7	70,0%				0
Boss	3	0	0,0%	1	0	0	0
Wanie - Arugula	1	1	100,0%				0
South Kivu	**3**	**1**	**33,3%**	0	0	0	0
Miti -Murhesa	3	1	33,3%				0

Machine Translated by Google

Bas Uélé	**13**	**4**	**30,8%**	0	0	0	0
Blind	1	1	100,0%				0
Viadana	6	1	16,7%				0
Claim	1	0	0,0%				0
Walk	5	2	40,0%				0
Sud Ubangi	**2**	**1**	**50,0%**	0	0	0	0
Hair	2	1	50,0%				0
Nord Ubangi	**3**	**1**	**33,3%**	1	0	0	0
You are logged in.	3	1	33,3%	1	0	0	
Total	**9637**	**4646**	**48,2%**	**85**	**29**	**15**	**44**

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA : Not Applicable.

Table 3. Status of alerts notified as of October 19, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	149	6	155	49	5	100	1	54	5
Bas Uélé	185	3	188	20	2	165	1	22	0
Ituri	1 119	77	1 196 139		74	980	3	210	122
North Kivu	1 397	59	1 456 192		59	1 205	0	231	142
Sud Ubangi ND ND ND ND					ND	ND	ND	ND	ND
Nord Ubangi	1	0	1	1	0	0	0	1	1
Boss	186	2	188	13	2	173	0	15	2
Total General	**3 037 147**	**3 184 414**			**142 2 623**		**5**	**533**	**272**

Machine Translated by Google

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

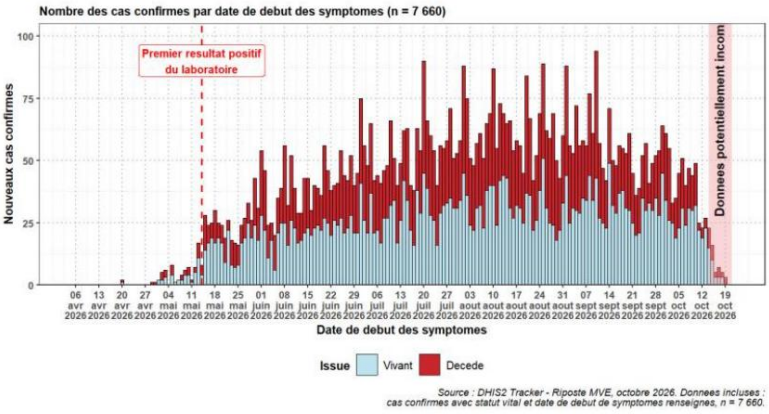
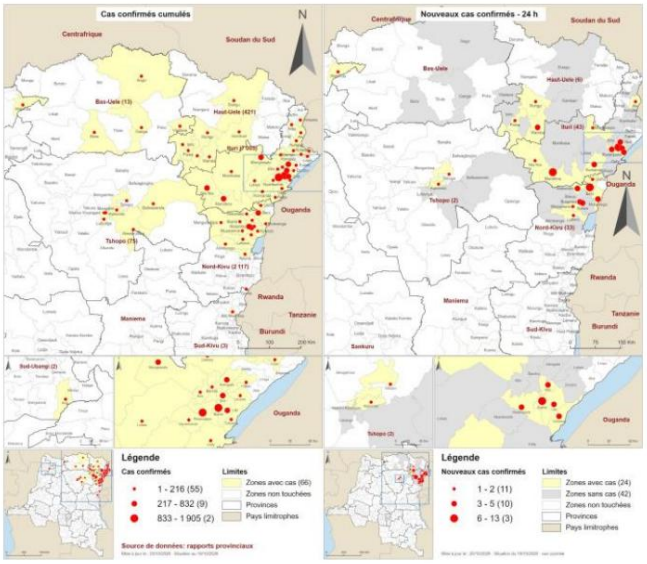


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1.COORDINATION

1.1. Main Actions

- Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

- During the day of October 19, 2026, 3,184 alerts were notified in the six provinces that submitted their data. All were verified and classified (100.0%), of which 556 (17.5%) were validated as suspected cases and 2,628 (82.5%) were invalidated. Of the 556 validated suspected cases, 533 (95.9%) were investigated, of which 272 (51.0%) were transferred to CT/CTE.

- Of the 31,725 contacts under follow-up in the seven provinces that submitted their data, 24,679 were seen in the last 24 hours, representing a follow-up rate of 77.8%, up from the previous day (76.9%) but still below the required threshold of 95.0%. This rate reached 100.0% in South Ubangi (44/44) and North Ubangi (61/61), 99.7% in Bas-Uélé (741/743), 93.0% in Haut-Uélé (1,576/1,694), and 86.5% in Tshopo (1,041/1,204). It remains insufficient in North Kivu (79.7%; 15,394/19,316).

2.1.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.1.3. Main actions

- For the day of October 19, 2026, 8 alerts were notified to the PoE/PoC of the five monitored provinces: 3 in Ituri, 3 in Tshopo, 1 in South Kivu and 1 in Haut-Uélé, the validated alerts were investigated and transferred to the CTE/CT.

Machine Translated by Google

Table 4. Status of key PoE/PoC indicators by province, as of October 19, 2026

Indicators	Ituri	North Kivu	South Kivu	Boss	Haut-Uélé	Global
Report completeness	98,4% (61/62)	95,2% (20/21)	55,6% (5/9)	55,6% (10/18)	100,0% (10/10)	88,3% (106/120)
% of functional strategic PoE/PoC	100%	100%	ND	100%	80%	ND
Number of PoE/PoC people upgrades	153 516	84 016	7 553	12 710	26 138	283 933
% of travelers screened	97,8%	96,1%	98,5%	97,9%	99,4%	97,5%
% of travelers who washed their hands	97,6%	96,1%	98,5%	97,4%	99,5%	97,3%
% of travelers made aware	99,6%	96,1%	100,0%	100,0%	100,0%	98,6%
Number of alerts notified	3	0	1	3	1	8
Number of validated live alerts (suspected cases)	2	0	0	0	1	3
Number of bodies recovered today	1	0	1	0	0	2
% of suspected cases transferred to the CTE/CT	50,0%	THAT	THAT	THAT	100,0%	66,7%
% of lifeless bodies swabed/secured	100,0%	THAT	0,0%	THAT	THAT	50,0%
% of suspected cases investigated within 4 h	100,0%	THAT	THAT	THAT	100,0%	100,0%

2.1.4. Challenges

- **In Ituri,** the lack of mobility for supervision in the health zones of Rwampara, Nizi and Bunia, the delay in payment of service providers for the month of September and the need for an ambulance to serve the PoCs of the Mandima health zone persist, and the Avakubi PoC remains non-functional due to insecurity.
- **In North Kivu,** completeness stands at 95.2% (20/21); the Butuhe PoC remains closed since the security incident of September 26, 2026, alert cells are not responding promptly to PoC alerts and the security threat on the Butembo axis, shortages of IPC and PPE supplies, and the lack of an ambulance dedicated to PoCs persist. • **In Tshopo,** completeness is only 55.6% (10/18). • **In South Kivu,** completeness remains low (55.6%; 5/9), and the Kavimvira,

Kalundu Port SNCC, and Baraka PoEs

do not have an isolation device.

- **In Bas-Uélé,** 4 out of 20 PoCs did not report (Garre Dingila, Mangite, Nawiwi and PK5 Dakwa) and the services of the PNHF, the training and motivation of service providers, the water supply, mobility, connectivity and the digitalization of PoE/PoC are lacking.

2.2. Laboratory

2.2.1. Main actions

- **Ituri: 43 new positive results** (28 living and 15 dead) out of 278 samples received and analyzed (positivity: 15.5%).
- **North Kivu: 33 new positive results** (22 living and 11 deaths) out of 199 samples received and analyzed (positivity: 16.6%).
- **Haut-Uélé: 6 new positive results** (4 living and 2 deceased) out of 24 samples received and analyzed (positivity: 25%).
- **Tshopo: 2 new positive results** (1 living and 1 deceased) out of 7 samples received and analyzed (positivity: 28,6%).
- **Bas Uélé :** 2 new samples received and analyzed, all came back negative.
- **North Ubangi: 1 new positive result** (1 positive) out of 4 samples received and analyzed (positivity: 25%)

Machine Translated by Google

2.3. Infection Prevention and Control (IPC/EDS)

2.3.1. Main actions

- **Ituri :** 29 out of 38 rings were opened (76.3%), 34 out of 39 households were decontaminated (87.2%), and 11 out of 11 health facilities were decontaminated (100.0%). Out of 115 EDS alerts (including 6 postponements), 85 EDS were carried out (73.9%), and 86 out of 104 bodies were swabed (82.7%).
- **Tshopo :** 9 ESS (Emergency Health Services) were monitored in 5 health zones, and 159 out of 159 service providers were briefed, including 10 hygiene workers in Bafwasende; 2 households were decontaminated (Makiso-Kisangani and Kabondo), and the Kwetu Health Center was provided with IPC (Infection Prevention and Control) kits. Of 6 EDS (Emergency Health Surveillance) alerts and 3 reports, 1 EDS was carried out and 1 body was swabned.
- **Haut-Uélé :** 2 out of 3 rings were opened (66.7%) and 21 out of 21 households were decontaminated (100.0%) in Wamba, and 5 out of 5 ESSs were decontaminated (100.0%) in Isiro, Wamba and Pawa; out of 4 EDS alerts, 2 EDS were carried out (50.0%) and 2 bodies were swabed, in Wamba and Pawa
- **North Kivu :** 9 out of 22 rings were opened (40.9%), 10 out of 12 health centers were decontaminated (83.3%), and 8 out of 15 households (53.3%) were treated. None of the 8 decontaminated households received kits. 15 infection prevention and control (IPC)/water, sanitation, and hygiene (WASH) assessments were conducted in 14 health centers (average score of 36.3%). Of the 53 emergency health alerts received and 40 reports from the previous day, 48 EHS were carried out and 52 bodies were swabed.
- **Bas-Uélé :** 9 ESS were monitored in 4 ZS, 42 out of 45 providers (93.3%) and 22 out of 22 hygienists were briefed and the DLM of the HGR of Buta (13/13), of the CS Makala (3/3), of the CM Mangindanginda (1/1) and of the CS of Congo (1/1) are functional; out of 3 EDS alerts, 2 bodies were swabed.

2.3.2. Challenges

- **In Ituri,** 9 out of 38 rings remain to be opened, 5 out of 39 households have not been decontaminated, 21 EDS failures have been recorded and 18 out of 104 bodies (17.3%) could not be swabed.

In North **Kivu,** 13 out of 22 treatment centers remain to be opened, notably in Katwa and Beni; 7 out of 15 households have not been decontaminated; no households have received kits (0/8); and 44 mass casualty incidents (MSIs) have been postponed until the following day, as one body could not be swabed in Vuhovi due to community resistance. In Tshopo, the shortage of infection prevention and control (IPC) supplies and equipment, as well as trained IPC providers, persists; the water supply to health centers remains problematic; and 5 out of 6 MSIs could not be carried out due to the absence of families. In Bas-Uélé, no MSIs were carried out for 3 alerts; 16 out of 20 priority health centers lack functional triage; none have mattresses; 17 out of 20 lack sufficient water storage for 72 hours; and the reporting of community deaths remains low. In Haut-Uélé, 2 families refused the EDS and the swab in Isiro, only 4 out of 7 health zones submitted their PCI report, and PCI kits for ESS and pillar mobility are lacking.

2.4. Holistic care

2.4.1. Main actions

- **In Ituri,** 83 new admissions were recorded, compared to 81 discharges, including 5 recoveries. Thus, 431 patients are hospitalized (193 confirmed and 238 suspected), representing an occupancy rate of 42.5%, including 57 critically ill patients; 34 patients were evacuated by ambulance, 411 out of 420 patients (97.9%) received hot meals, and 27 units of blood were distributed in 4 Ebola Treatment Centers (ETCs).
- **In North Kivu,** 154 new admissions were recorded, compared to 95 discharges, including 10 recoveries. Thus, 509 patients are hospitalized, including 58 children under 5 years old, and 3 patients are in critical condition. The overall occupancy rate of Ebola Treatment Centers (ETCs) is 81.0% (332/410), 86.5% for suspected cases (166/192), and 76.1% for confirmed cases (166/218). 177 patients (34.8%) are being treated in specialized healthcare facilities (ESS) that are not ETCs. 22 ambulance trips were successfully completed (9 out of 14 ambulances used), and 129 confirmed cases, 172 suspected cases, 404 people with HIV/AIDS (PPIs), and 216 caregivers received hot meals.
- **In Tshopo,** 3 new admissions of suspected cases were recorded (1 at the Météo Treatment Center and 2 at the Bafwasende Treatment Center) compared to 2 discharges of recovered patients. Thus, 13 patients are hospitalized out of 36 beds, representing an occupancy rate of 36.1%.
- **In Bas-Uélé,** 20 new admissions were recorded and the patients were isolated without discharge, carrying with 47 patients in isolation, the 3 beds for confirmed cases being occupied (100.0%).
- **In Haut-Uélé,** 4 new admissions were recorded against 28 discharges, including 7 recoveries in Pawa, 1 escape and 1 transfer to the HGR; 29 patients are hospitalized (9 confirmed and 20 suspected) for 169 beds, representing an occupancy rate of 17.2%, including 6 critical patients, and 31 patients, 55 companions and 70 PPL received hot meals.

Machine Translated by Google

2.4.2. Challenges

- **In Ituri,** the low capacity of some CTE/CT (Kilo, Lolwa, Mandima and Biakato), the care of 57 critical patients, the insufficient quantity of blood in relation to the needs of the CTE and hospitals, the lack of food for 9 patients in the Mandima health zone (CT Mandima and CT Makeke) and the low reporting of ambulance movements (17/33) remain worrying.

- **In North Kivu,** 86.5% of suspected cases are occupied (166/192), 34.8% of hospitalized patients (177/509) are being treated outside of CTE/CT and data completeness is only 86% (Kyondo and Vuhovi missing).

- **In Tshopo,** the lack of food and nutritional support for PPLs in Bafwasende and the 4 confirmed cases in Bafwasende who have been lost to follow-up remain a concern.
- **In Bas-Uélé,** the delay in establishing the standardized Ebola Treatment Center (ETC) at the Buta General Referral Hospital (HGR), the isolation of 44 suspected cases in emergency shelters (ESS) without nutritional support, and the delay in providing psychosocial support to the 3 confirmed cases remain major challenges. In Haut-Uélé, the poor network coverage of some ETCs, staff demotivation, insufficient nutritional supplies (F-75, F-100, Plumpy'Nut), and inadequate water and electricity access to some ETCs persist.

2.5. Vaccination

2.5.1. Main actions

- **In Ituri,** vaccination of people living with HIV (PLHIV) using Ervebo and research activities on the efficacy of the BBDV vaccine have been paused pending validation by the research committee; the Partners, EBO-PEP, and Oracle trials are continuing, and the TUMAINI project research teams are undergoing training. The cumulative number of vaccinated PLHIV remains at 1,260.

In North **Kivu,** 150 people living with HIV (PLHIV) were vaccinated in the Beni health zone, bringing the total to 472 out of a target of 7,105 (6.6%). 128 doses remain available out of the 600 received, and one non-serious case of acute liver injury (ALI) was reported. In Bas-Uélé, 500 doses of Ervebo vaccine were received, a compassionate vaccination team is being prepared for the Ango health zone, and coverage has reached 11.4% of the target (1,500/13,202). In Tshopo, eight rental vehicles were made available to the Expanded Program on Immunization (EPI) for vaccination. In Haut-Uélé, the PLHIV vaccination campaign was completed with a total of 589 PLHIV vaccinated (470 in Isiro, 80 in Pawa, and 39 in Wamba), and payment for the two vaccination teams in Isiro has been initiated.

2.5.2. Challenges

- **In Ituri,** the suspension of vaccination of PPLs awaiting validation and the target of PPLs and TPLs still to be redefined are hindering the calculation and progress of vaccination coverage.
- **In North Kivu,** the coverage of the PPL remains low (6.6% in Beni, out of a provincial target of 36,070 PPL in 15 ZS) and the vaccine transport devices (ARKTEK) are still to be confirmed with the WHO; in Bas-Uélé, the UCC is not operational; in Tshopo, the daily vaccination data could not be used; in Haut-Uélé, the BCG vaccine has been out of stock at the EPI station in Isiro since September 25, 2026 and the ultra-cold refrigerators are out of order.

2.6. Continuity of Care

2.6.1. Main actions • In North

Kivu, 5 out of 5 separated children were taken into care at the nursery, 16 out of 16 separated or orphaned infants are on breast milk substitute, 1 out of 1 malnourished child was taken into care, 8 confirmed unstable children benefited from specific diets and 12 households received food; the evaluation of CH Kyaghala reveals 4 out of 108 providers trained in PCI and a shortage of medication for the management of TB/HIV.

- **In Tshopo,** the evaluation of the COKIS CS shows 16 out of 20 agents vaccinated against Ebola (80.0%) and 8 out of 20 trained (40.0%), with effective triage and a functional incinerator; nutritional inputs (Plumpy'Nut, F-75, F-100) have been received and the recovery plan of the CM Headquarters is underway.
- **In Bas-Uélé,** the evaluation of the Ganga General Referral Hospital (HGR) shows a decrease in the service utilization rate, from 29.5% in June to 20.3% in September 2026, compared to a standard of 60%, reflecting low patient attendance or delayed access to care. In Haut-Uélé, the evaluation of the Bolebole Hospital (Wamba) reveals low service utilization due to fear of infection, missed appointments for routine vaccinations and chronic disease management, insufficient latrines and showers, and a 55% shortage of tracer medications; 3 people with primary AIDS (PPL) are being monitored in the Wamba and Boma Mangbetu treatment centers.

Machine Translated by Google

- **In Ituri,** 75 children received nutritional support from LNPE/SLM: 11 out of 11 at the CTE, 4 out of 4 at the nursery and 60 out of 60 in the community; the second phase of mass administration of antimalarials was launched and routine vaccines were deployed in the health zones of Kilo, Damas, Tchomia, GethyN and Boga.

2.7. Risk Communication and Community Engagement (RCCE)

2.7.1. Main actions

- **In Ituri,** 18,413 home visits reached 63,612 people; 202 educational talks brought together 5,306 people, 61 mass awareness campaigns reached 8,428 people, 2 community debates brought together 47 people, and 4 advocacy sessions brought together 29 religious and community leaders (Rwampara, Mandima, and Komanda). A total of 312 community alerts, including 139 deaths, were reported; 36 out of 42 feedback points (85.7%) were resolved within 48 hours, and the single incident reported in Komanda remains unresolved. The CREC facilitated the pre-listing of 191 contacts, the decontamination of 11 out of 11 households, the completion of 15 out of 19 DHS (78.9%), the announcement of 24 out of 24 positive results, and the transfer of 11 out of 11 suspected cases to the Ebola Treatment Center (ETC). The completeness of CREC reports remains low (9/28 ZS, 32.1%).

- **In North Kivu,** 9,013 home visits reached 28,701 people, 297 educational talks reached 6,579 people, 45 mass communications reached 7,033 people, and 1 briefing reached 34 people; 191 community alerts were reported out of 650 expected (29.4%), 17 out of 25 feedback requests were received (68.0%), and 2 out of 5 incidents (40.0%) were resolved within 48 hours. 1,718 out of 4,750 people (36.2%) participated in community actions. The "STOP EBOLA" day was marked by a caravan in Beni and a broadcast on more than 72 media outlets, and 217 taxi drivers received awareness training in Musienene.

- **In North Ubangi,** 323 students were sensitized in Ngama, and advocacy was conducted with religious leaders in Gbadolite. Community outreach included 39 home visits reaching 234 people, 14 talks reaching 232 people, and 4 mass awareness sessions reaching 1,231 people. 311 people were sensitized on the Education for Health and Social Development (EHS) in Isiro and Wamba. 39 community alerts, including 4 deaths, were reported. 11 out of 13 feedback points (84.6%) were resolved, and the 2 reported incidents (the escape of a body from the Libiki Hospital and the death in Rungu of a confirmed escapee from Isiro) remain under investigation.

2.8. Mental Health and Psychosocial Support (MHPSS)

2.8.1. Main actions

- **In Ituri,** 30 out of 42 confirmed cases (71.4%), 87 out of 144 confirmed cases (60.4%), 54 out of 83 new suspected cases (65.1%), and 121 out of 139 suspected cases (87.1%) received SMSPS interventions. The teams supported 329 caregivers and 79 PPLs, monitored 36 out of 41 recoveries (87.8%) in the community and 10 out of 10 children in daycare, assisted 7 out of 11 separated and orphaned households, reintegrated 2 out of 5 recoveries, and announced 49 results, 30 of which were positive; the training of 12 psychologists and APS/TPS workers in Mandima was completed.

- **In North Kivu,** 30 out of 48 new cases (62.5%) benefited from SMSPS interventions, 212 confirmed cases and 215 suspected cases are being monitored, 37 out of 40 households with separated and orphaned children (92.5%) received support, 14 recovered individuals were reintegrated, and 132 test results were announced, including 48 positive results. In Tshopo, 3 confirmed cases received psychological support, 6 suspected cases and 404 individuals received psychoeducation, 7 people with HIV/AIDS received support, and 2 out of 3 resistance mechanisms were overcome.
- **In Haut-Uélé,** 4 new confirmed cases, 10 confirmed cases under follow-up, 5 new suspected cases, and 20 suspected cases under follow-up received SMSPS interventions; 58 caregivers, 61 people living abroad, and 36 household members received support; 13 separated children were monitored in the community; and 26 test results were announced, including 3 positive results. In Bas-Uélé, only 1 new suspected case received an SMSPS intervention; no other indicators were reported.

2.8.2. Challenges

- **In Ituri,** only 14 of the 28 affected health zones are covered (50.0%) and 12 of the last 42 confirmed cases (28.6%) have not yet been taken care of; the deployment of psychologists and APS/TPS trained in Mandima is expected.

- **In Bas-Uélé,** the psychosocial support for the 3 confirmed is lagging behind; in Tshopo, the APS of Wanie-Rukula still need to be trained and the pillar lacks communication units and motorcycles; in Haut-Uélé, the pillar lacks computer equipment, motorcycles for the APS of Wamba and Boma Mangbetu and nutritional kits for weaned infants in Pawa and Wamba.

Machine Translated by Google

2.9. Logistics

2.9.1. Main actions.

- Ensure the mobility of all field teams in the affected provinces; in North Kivu, the construction of the cabins and the passenger walkway at Beni airport continues; in Tshopo, the construction sites of the Bafwasende CTE, the extension of the Météo CTE and the Kongakonga isolation site are being monitored, while input stocks are very low (25 kg of chlorine for a need of 1,800 kg); in Bas-Uélé, the construction of the Buta HGR CTE continues, but the diesel stock is exhausted.

2.10. Security

2.11. Main actions.

- **In North Kivu,** the "STOP EBOLA" caravan and the EDS (Emergency Response Teams) of 6 confirmed cases were escorted to Beni. • **In Tshopo,** attacks and threats against intervention teams have been reported, and a confirmed case has been identified. No. 53 always wanders in the community;
- **In Ituri,** the Avakubi Point of Control (PoC) remains non-functional due to insecurity, and resistance persists in Mongbwalu, Fataki, and Komanda. • **In Haut-Uélé,** a body was removed by the family from the Libiki Hospital after they refused to perform a swab, and a confirmed escapee from Isiro died in AS Bala (Rungu).

2.12. PSEA (Prevention of Sexual Exploitation and Abuse)

2.12.1. Main actions

- **In Ituri,** 33 service providers from the CTE in Lita, the HGR and subcontractors (13 women and 20 men) were briefed on the PSEAH and signed the code of good conduct in Bunia, and 62 beneficiaries of the CPS were sensitized;
- **In North Kivu,** 60 staff members (38 men and 22 women) were briefed and 254 people awareness raised in the community, particularly in Katwa;
- **In Haut-Uélé,** 97 Red Cross agents (60 men and 37 women) and 18 members of DIVIGENRE were made aware of Isiro and 105 codes of good conduct were signed.

2.12.2. Challenges

- At the national level, coverage of affected areas remains low (56 health zones out of 151 with PSEA activities, i.e. 37.1%), only 1 out of 14 partners reports their activities, visibility supports are insufficient and the effectiveness of CBCM is low; in Ituri, only 15 out of 36 health zones (41.7%) know the reporting mechanism, compared to 5 out of 34 (14.7%) in North Kivu; in Haut-Uélé, the leaflets received are in Swahili while 10 out of 13 health zones are Lingalophone and the pillar lacks computer kits.

MESSAGES CLES

ÿ In the past 24 hours, 533 out of 556 suspected cases (95.9%) have been investigated in the provinces that submitted their data.
However, 23 suspected cases remain uninvestigated (20 in North Kivu and 3 in Ituri). Furthermore, the contact tracing rate has improved slightly, rising from 76.9% the previous day to 77.8%, but remains below the required threshold of 95.0%, with a critical situation in Bunia (38.8%). This situation necessitates strengthening epidemiological surveillance and contact tracing to limit the risks of community transmission.

ÿ Maintain a level of vigilance regarding epidemiological surveillance aspects, consolidate IPC and CREC actions at the community level while providing quality care in dedicated care structures with adequate reception capacities.

TIMELINE OF KEY EVENTS

ÿ From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.

ÿ Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.

Machine Translated by Google

TASK FORCE PRESIDENTIAL EBOLA 17
Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
MVE17 Incident Management System

Director General of INSP
Dr. MWAMBA KAZADI Dieudonné

Email: dieudonne.mwamba@insp.cd **Incident**
Manager: Professor
Dr. AHUKA MUNDEKE Steve
Email: steve.ahuka@unikin.ac.cd

Operations Manager
Dr. KOBBE NGWAMA John

Editorial Team
Professor Dr. MULANGU KATULUMBA Félix-Médard
Dr. KATUSI BISELENGE Jules
Mrs. WISE DEAD Marie
Dr. DIUR MANANASHI Floryan
Mr. Kamuina Kebela John

Contact :
www.insp.cd
contact@sgiebola17.org

Freephone number: 151. www.sante.gouv.cd